

THE HORMONE RESET SERIES

Testosterone for Women

*The Hormone You Weren't Told About —
and Why It Matters*

Disclaimer

This book is intended for educational and informational purposes only. The content does not constitute medical, clinical, or professional health advice. The author and publisher are not licensed healthcare providers.

Nothing in this book should replace consultation with a qualified physician, endocrinologist, gynecologist, or other healthcare professional. Hormone health is highly individual and requires personalized clinical evaluation.

Always seek the advice of your doctor or qualified health provider with questions you may have regarding a medical condition or before starting any new health regimen. Never disregard professional medical advice or delay seeking it because of something you have read in this book.

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Women Have Testosterone Too

Testosterone is not a male hormone. It's a human hormone that happens to be present in different amounts.

What It Does in the Female Body

Women produce testosterone in the ovaries and adrenal glands in amounts roughly 10–20 times lower than men — but this doesn't make it inconsequential. In women, testosterone influences libido, mood, energy, muscle maintenance, bone density, cognitive sharpness, and motivation.

Symptoms of low testosterone in women include: diminished sexual desire (one of the most commonly reported but least commonly investigated complaints in women's health), fatigue that doesn't respond to sleep, loss of muscle mass and strength, reduced motivation or sense of drive, and blunted mood.

These symptoms overlap with many other conditions — depression, thyroid disorders, perimenopause — which is why testosterone is often missed as a contributing factor.

*"Low testosterone in women is common and
commonly overlooked."*

When Testosterone Is Too High

Elevated androgens in women produce their own distinct set of challenges.

PCOS and Androgen Excess

Polycystic ovarian syndrome affects an estimated 10% of women of reproductive age and is the most common cause of androgen excess. Elevated testosterone in PCOS drives acne, hirsutism (unwanted facial and body hair), scalp hair thinning, and irregular or absent periods.

PCOS is a complex condition with significant metabolic overlap — insulin resistance both drives and worsens androgen excess in many PCOS phenotypes.

Other causes of elevated androgens include congenital adrenal hyperplasia, adrenal or ovarian tumors (rare), and certain medications. Any new-onset rapid androgen excess warrants medical evaluation.

"Androgen imbalance in either direction has quality-of-life consequences worth addressing."

Testing and Interpretation

Testosterone testing in women is more complex than a single number suggests.

What to Test and Why

Total testosterone provides a gross measure. Free testosterone — the unbound, biologically active fraction — is more clinically relevant. Sex hormone binding globulin (SHBG) affects how much testosterone is free versus bound, and elevated SHBG (common with oral contraceptive use) can lower free testosterone significantly.

Testing should be done in the morning and, for cycling women, during the same phase of the cycle for meaningful comparison. Results should be interpreted by a clinician familiar with female reference ranges — not those developed primarily from male physiology.

"Context and clinical correlation matter more than a single number."

Supporting Healthy Testosterone Levels Naturally

Lifestyle factors influence androgen levels in both directions.

Lifestyle Foundations

Resistance training consistently and reliably increases testosterone in women — this is one of many reasons strength training is repeatedly highlighted as a cornerstone intervention across female hormone health.

Adequate dietary fat is required for sex hormone synthesis. Very low fat diets reliably suppress testosterone (and other steroid hormones). Healthy fats from avocados, olive oil, eggs, and fatty fish support the lipid substrate hormones are built from.

Zinc is required for testosterone synthesis. Chronic zinc deficiency impairs androgen production. Red meat, shellfish (especially oysters), pumpkin seeds, and legumes are dietary sources.

"You can't build sex hormones without fat and zinc."

Testosterone Therapy for Women

Low-dose testosterone therapy for women is a growing area of clinical evidence — and still underutilized.

The Current Evidence

The strongest evidence for testosterone therapy in women exists for hypoactive sexual desire disorder (HSDD) — low sexual desire that causes distress. Multiple randomized controlled trials show benefit from low-dose testosterone.

Evidence also supports benefit for fatigue, mood, and muscle maintenance in postmenopausal women, though this remains less established.

Testosterone therapy in women is typically administered as a low-dose topical gel or cream. It is not FDA-approved for women in the US, but is prescribed off-label by practitioners familiar with female hormone health. This requires an honest conversation about the evidence, the risks, and the monitoring required.

"The evidence for testosterone therapy in women is growing. Find a provider who knows it."

Androgens Across the Lifespan

Testosterone in women follows its own lifecycle, with peaks, declines, and clinical implications at each stage.

The Trajectory

Peak testosterone in women occurs in the mid-20s and declines gradually and continuously thereafter. By menopause, women have approximately half the testosterone they had at their peak.

Surgical menopause (oophorectomy — removal of the ovaries) causes an abrupt 50% reduction in testosterone production and often produces more acute low-androgen symptoms than natural menopause.

Oral contraceptives increase SHBG, which binds testosterone and reduces free levels — some women experience significant mood, libido, and energy changes on certain OCP formulations that are partially driven by this mechanism.

Understanding the testosterone trajectory across your life informs conversations about energy, motivation, and wellbeing that might otherwise be attributed to 'just

getting older.'

"Androgen health is part of lifelong hormone health, not just a reproductive-years concern."
